Clinical Picture and Etiology of Anxiety Disorders

Anxiety and Fear

Anxiety disorders are characterized by **clinically significant fear and/or anxiety**.

- **Anxiety** → A general feeling of apprehension about possible future danger.
- **Fear** → An alarm reaction in response to immediate danger.

According to **DSM-5**, anxiety disorders are grouped together because they share symptoms of excessive fear and anxiety and related behavioral disturbances.

Fear and Anxiety: Response Patterns

Many researchers propose a fundamental distinction between **fear** and **anxiety** response patterns.

Fear Response Pattern: Fear is considered a **basic emotion**, shared by many animals.

1. Nature of Fear

- Immediate reaction to **imminent threat**
- Activates the **fight-or-flight response** of the autonomic nervous system
- Almost instantaneous alarm reaction
- Adaptive value: facilitates escape from danger

Examples: Dangerous predator, Someone pointing a loaded gun

2. Panic Attack and Fear: When fear occurs **without obvious external danger**, it is termed as **Spontaneous (uncued) panic attack**

Symptoms of a panic attack are nearly identical to fear, but panic often includes additional cognitive symptoms:

- Subjective sense of impending doom
- Fear of dying
- Fear of "going crazy"
- Fear of losing control

These cognitive features typically do not occur in ordinary fear states.

3. Components of Fear and Panic

Fear and panic consist of three components:

(a) Cognitive / Subjective

- "I feel afraid."
- "I'm going to die."

(b) Physiological

- Increased heart rate
- Rapid breathing
- Sweating
- Autonomic nervous system activation

(c) Behavioral

- Strong urge to escape or flee

Anxiety Response Pattern: Anxiety differs from fear in several important ways.

1. Nature of Anxiety

- More future-oriented
- More diffuse
- Complex blend of unpleasant emotions and cognitions
- Not tied to immediate danger

2. Components of Anxiety: Like fear, anxiety includes cognitive, physiological, and behavioral components.

(a) Cognitive / Subjective Components

- Negative mood
- Worry about possible future threats
- Self-preoccupation
- Sense of unpredictability
- Perceived inability to control future events

Example thought: "Something awful may happen, and I won't be able to handle it."

(b) Physiological Components

- Chronic tension
- Hyperarousal
- Muscular tightness
- Readiness for potential danger

Unlike fear:

- Anxiety does **not** activate the full fight-or-flight response.
- Instead, it **primes** the body for possible danger.

(c) Behavioral Components

- Tendency to avoid potential threat situations
- No immediate urge to flee

3. Functional Differences Between Fear and Anxiety

Fear	Anxiety
Immediate threat	Future-oriented threat
Fight-or-flight activation	Tension and preparedness
Urge to flee	Avoidance tendency
Alarm response	Risk assessment and vigilance

Adaptive Value

1. Mild to moderate anxiety enhances: Learning, Planning, Performance

Example: Moderate exam anxiety improves preparation.

2. Chronic and severe anxiety becomes maladaptive.
3. Leads to impairment seen in anxiety disorders.

Learning and Conditioning of Fear and Anxiety

Many fears are **learned**. This has been shown through research in humans and animals which have found that fear and anxiety are highly **conditionable**.

The classical conditioning model shows that previously neutral stimuli (Conditioned Stimulus – CS) when paired with frightening event (Unconditioned Stimulus – US)
→ Neutral stimulus begins to elicit fear/anxiety (Conditioned Response – CR)

Example: Angela witnessed her father physically abusing her mother.

- Father's abuse = Unconditioned stimulus
- Fear response = Unconditioned response
- Sound of father's car in driveway (initially neutral)
  → Becomes conditioned stimulus
  → Triggers anxiety

Eventually:

- Even thinking about her father elicits anxiety.
- Thoughts and mental images can function as conditioned stimuli.

Anxiety Disorders (DSM-5 Classification)

Anxiety disorders are characterized by:

- Unrealistic or irrational fears and anxieties
- Disabling intensity
- Significant distress or impairment

They cause:

- Personal suffering
- Economic burden
- Health care costs

Anxiety Disorders Recognized in DSM-5

DSM-5 classifies the following under **Anxiety Disorders**:

1. **Specific Phobia**
2. **Social Anxiety Disorder (Social Phobia)**
3. **Panic Disorder**
4. **Agoraphobia**
5. **Generalized Anxiety Disorder**
6. **Separation Anxiety Disorder**
7. **Selective Mutism**

Disorders No Longer Classified Under Anxiety Disorders in DSM-5

Unlike DSM-IV-TR, DSM-5 separates the following:

- **Obsessive-Compulsive Disorder** → now under *Obsessive-Compulsive and Related Disorders*
- **Posttraumatic Stress Disorder (PTSD)** → now under *Trauma- and Stressor-Related Disorders*
- **Acute Stress Disorder** → also under *Trauma- and Stressor-Related Disorders*

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1. PHOBIC DISORDERS (DSM-5)

Phobic disorders are characterized by **persistent, excessive, and disproportionate fear** of specific objects, situations, or activities that pose little or no actual danger. The fear leads to **significant avoidance**, distress, or functional impairment.

Under **DSM-5**, phobic disorders include:

1. **Specific Phobia**
2. **Social Anxiety Disorder (Social Phobia)**
3. **Agoraphobia**

I. SPECIFIC PHOBIA

A. Key / Core Characteristics

- Marked and persistent fear of a specific object or situation.
- Fear is excessive or out of proportion to actual danger.
- Immediate anxiety response upon exposure (often resembles a panic attack but with a clear trigger).
- Fear persists for **6 months or more** (DSM-5 requirement).
- Causes significant distress or impairment in functioning.

criteria for
Specific Phobia

DSM-5

A. Marked fear or anxiety about a specific object or situation (e.g., flying, heights, animals, receiving an injection, seeing blood).
Note: In children, the fear or anxiety may be expressed by crying, tantrums, freezing, or clinging.

B. The phobic object or situation almost always provokes immediate fear or anxiety.

C. The phobic object or situation is actively avoided or endured with intense fear or anxiety.

D. The fear or anxiety is out of proportion to the actual danger posed by the specific object or situation and to the sociocultural context.

E. The fear, anxiety, or avoidance is persistent, typically lasting for 6 months or more.

F. The fear, anxiety, or avoidance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

G. The disturbance is not better explained by the symptoms of another mental disorder, including fear, anxiety, and avoidance of situations associated with panic-like symptoms or other incapacitating symptoms (as in agoraphobia); objects or situations related to obsessions (as in obsessive-compulsive disorder); reminders of traumatic events (as in posttraumatic stress disorder); separation from home or attachment figures (as in separation anxiety disorder); or social situations (as in social anxiety disorder).

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Common Types (DSM-5 Specifiers)

- Animal type (e.g., dogs, snakes)
- Natural environment type (e.g., heights, storms)
- Blood-injection-injury type
- Situational type (e.g., elevators, flying)
- Other type (e.g., choking, vomiting)

B. Behavioural Elements

- Active avoidance of phobic stimulus.
- Avoidance of representations (photos, videos, TV images).

- Enduring situations only with intense distress.
- Safety behaviors (e.g., always sitting near exit, carrying water).
- Anticipatory anxiety before exposure.

Example: A person with claustrophobia avoids elevators, even if it causes occupational inconvenience.

C. Causes

1. Psychological Causes

- **Classical conditioning** (direct traumatic experience).
- **Vicarious learning** (observing others' fear).
- **Preparedness theory** (biological readiness to fear certain stimuli like snakes, heights).
- Cognitive biases (overestimation of danger).

2. Social Causes

- Parental modeling of fear.
- Overprotective parenting.
- Cultural transmission of fear beliefs.

3. Biological Causes

- Genetic vulnerability (moderate heritability).
- Amygdala hyperreactivity.
- Blood-injection-injury type may involve vasovagal response (fainting).

D. Treatment

1. Behavioral Treatment (Most Effective)

- **Systematic desensitization**
- **Graduated exposure (in vivo exposure)**
- Virtual reality exposure (for flying, heights)

2. Cognitive Techniques: Challenging exaggerated danger beliefs.

3. Medication

- Generally **not primary treatment**.
- May use short-term anxiolytics in rare cases (e.g., flying).

CASE STUDY: Refer to Pg 167 & 172 for case study.

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2. SOCIAL ANXIETY DISORDER (SOCIAL PHOBIA)

A. Key / Core Characteristics

- Marked fear of social or performance situations involving scrutiny.
- Fear of negative evaluation, embarrassment, or humiliation.
- Social situations almost always provoke anxiety.
- Fear is excessive and persistent (6 months or more).
- Causes significant distress or functional impairment.

criteria for
Social Anxiety Disorder (Social Phobia) — DSM-5

A. Marked fear or anxiety about one or more social situations in which the individual is exposed to possible scrutiny by others. Examples include social interactions (e.g., having a conversation, meeting unfamiliar people), being observed (e.g., eating or drinking), and performing in front of others (e.g., giving a speech).
Note: In children, the anxiety must occur in peer settings and not just during interactions with adults.

B. The individual fears that he or she will act in a way or show anxiety symptoms that will be negatively evaluated (i.e., will be humiliating or embarrassing; will lead to rejection or offend others).

C. The social situations almost always provoke fear or anxiety.
Note: In children, the fear or anxiety may be expressed by crying, tantrums, freezing, clinging, shrinking, or failing to speak in social situations.

D. The social situations are avoided or endured with intense fear or anxiety.

E. The fear or anxiety is out of proportion to the actual threat posed by the social situation and to the sociocultural context.

F. The fear, anxiety, or avoidance is persistent, typically lasting for 6 months or more.

G. The fear, anxiety, or avoidance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

H. The fear, anxiety, or avoidance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition.

I. The fear, anxiety, or avoidance is not better explained by the symptoms of another mental disorder, such as panic disorder, body dysmorphic disorder, or autism spectrum disorder.

J. If another medical condition (e.g., Parkinson's disease, obesity, disfigurement from burns or injury) is present, the fear, anxiety, or avoidance is clearly unrelated or is excessive.

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Subtypes (DSM-5)

- Performance only
- Generalized (most social situations)

B. Behavioural Elements

- Avoidance of public speaking, eating in public, social gatherings.
- Limited eye contact.
- Safety behaviors (rehearsing sentences mentally).
- Self-focused attention.
- Post-event rumination ("I looked foolish.").

C. Causes

1. Psychological Causes

- Classical conditioning (humiliation experiences).

- Cognitive distortions:
    - Overestimating negative evaluation.
    - Mind-reading bias.
- Self-focused attention model.
- Fear conditioning to angry/critical faces.

2. Social Causes

- Parental rejection or overcontrol.
- Social isolation during childhood.
- Peer teasing or bullying.
- Cultural emphasis on evaluation or performance.

3. Biological Causes

- Behavioral inhibition temperament.
- Genetic predisposition (moderate heritability).
- Amygdala hyperactivation to social threat.
- High neuroticism.

D. Treatment

1. Cognitive Behavioral Therapy (First-line)

- Exposure to social situations.
- Cognitive restructuring.
- Attention retraining.
- Video feedback techniques.

2. Medications

- SSRIs
- SNRIs
- MAOIs (less common today)
- Beta-blockers (for performance anxiety)

3. Combined Treatment

- CBT + medication in moderate to severe cases.

Case study: Refer to Pg 174 & 177

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3. AGORAPHOBIA

A. Key / Core Characteristics

Marked fear or anxiety about **two or more** of the following (DSM-5):

- Public transportation
- Open spaces
- Enclosed spaces
- Standing in line or crowds
- Being outside home alone

Fear is based on:

- Escape being difficult, or
- Help being unavailable in case of panic-like or incapacitating symptoms.

Symptoms persist for **6 months or more** and cause impairment.

criteria for
Agoraphobia

DSM-5

A. Marked fear or anxiety about two (or more) of the following five situations:

1. Using public transportation (e.g., automobiles, buses, trains, ships, planes).
2. Being in open spaces (e.g., parking lots, marketplaces, bridges).
3. Being in enclosed places (e.g., shops, theaters, cinemas).
4. Standing in line or being in a crowd.
5. Being outside of the home alone.

B. The individual fears or avoids these situations because of thoughts that escape might be difficult or help might not be available in the event of developing panic-like symptoms or other incapacitating or embarrassing symptoms (e.g., fear of falling in the elderly; fear of incontinence).

C. The agoraphobic situations almost always provoke fear or anxiety.

D. The agoraphobic situations are actively avoided, require the presence of a companion, or are endured with intense fear or anxiety.

E. The fear or anxiety is out of proportion to the actual danger posed by the agoraphobic situations and to the sociocultural context.

F. The fear, anxiety, or avoidance is persistent, typically lasting for 6 months or more.

G. The fear, anxiety, or avoidance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

H. If another medical condition (e.g., inflammatory bowel disease, Parkinson's disease) is present, the fear, anxiety, or avoidance is clearly excessive.

I. The fear, anxiety, or avoidance is not better explained by the symptoms of another mental disorder—for example, the symptoms are not confined to specific phobia, situational type; do not involve only social situations (as in social anxiety disorder); and are not related exclusively to obsessions (as in obsessive-compulsive disorder), perceived defects or flaws in physical appearance (as in body dysmorphic disorder), reminders of traumatic events (as in posttraumatic stress disorder), or fear of separation (as in separation anxiety disorder).

Note: Agoraphobia is diagnosed irrespective of the presence of panic disorder. If an individual's presentation meets criteria for panic disorder and agoraphobia, both diagnoses should be assigned.

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B. Behavioural Elements

- Avoidance of feared locations.
- Gradual expansion of avoidance.
- Reliance on companions ("safe person").
- Avoidance of activities that increase bodily arousal:
    - Exercise
    - Caffeine
    - Sexual activity
    - Scary movies

In severe cases: Person may be housebound.

C. Causes

1. Psychological Causes

- Often develops after panic attacks.
- Conditioning of external situations to panic sensations.
- Catastrophic misinterpretation of bodily sensations.
- Avoidance maintained by negative reinforcement.

2. Social Causes

- Early experiences of uncontrollability.
- Overprotective upbringing.
- Stressful life events.

3. Biological Causes

- Shared genetic vulnerability with panic disorder.
- Dysregulation in serotonin and norepinephrine systems.
- Amygdala hypersensitivity.

D. Treatment

1. Exposure Therapy (Primary Treatment)

- Gradual in vivo exposure.
- Elimination of safety behaviors.
- Interoceptive exposure (if panic-related).

2. Cognitive Therapy

- Challenging catastrophic beliefs.
- Reframing escape and helplessness beliefs.

3. Medications

- SSRIs
- SNRIs
- Short-term benzodiazepines (cautious use)

Case study: Refer to Pg 180

COMPARATIVE SUMMARY OF PHOBIC DISORDERS

Feature	Specific Phobia	Social Anxiety Disorder	Agoraphobia
Focus of Fear	Specific object/situation	Social scrutiny	Escape/help unavailability
Panic-like Response	Yes (triggered)	Yes (social trigger)	Often linked to panic
Avoidance	Object-specific	Social situations	Multiple public settings
Duration	≥ 6 months	≥ 6 months	≥ 6 months
First-line Treatment	Exposure	CBT + Exposure	Exposure

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4. Generalized Anxiety Disorder (GAD)

Key / Core Characteristics

- Excessive and persistent **anxiety and worry** about multiple life areas (work, health, finances, family, minor events).
- Occurs **more days than not for at least 6 months**.
- Worry is **difficult to control**.
- Anxiety is generalized ("free-floating") rather than limited to a specific object or situation.
- Causes **clinically significant distress or impairment** in social, occupational, or other functioning.

criteria for
Generalized Anxiety Disorder

DSM-5

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months):

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.
2. Being easily fatigued.
3. Difficulty concentrating or mind going blank.
4. Irritability.
5. Muscle tension.
6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

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Associated Symptoms (≥ 3 required in adults)

1. Restlessness or feeling keyed up/on edge
2. Easily fatigued
3. Difficulty concentrating or mind going blank
4. Irritability
5. Muscle tension
6. Sleep disturbance

Behavioural Elements

- Constant reassurance seeking (e.g., calling loved ones frequently).
- Procrastination and over-preparation.
- Repeated checking behaviors.
- Avoidance of potentially stressful tasks.
- Hypervigilance for possible threats in the environment.
- Difficulty relaxing; inability to tolerate uncertainty.

Causes

A. Biological Factors

- **Genetic predisposition**:
  - Tends to run in families.
  - Heritability estimates around **30–40%** (twin studies).
- **Neurotransmitter imbalance**:
  - **Serotonin** dysregulation affects mood and anxiety regulation.
  - Reduced **GABA** activity leads to increased neural excitability and persistent worry.

- Brain areas involved: **Amygdala** (fear processing) and **prefrontal cortex** (worry regulation).

B. Psychological Factors

- Maladaptive beliefs about worry ("Worrying prevents bad things from happening").
- Cognitive distortions (catastrophizing, overestimation of threat).
- Low tolerance for uncertainty.
- Learned patterns of chronic worry as a coping strategy.

C. Social / Environmental Factors

- Chronic life stress (financial strain, family conflict).
- Overprotective or anxious parenting models.
- Early exposure to unpredictable or stressful environments.

Treatment

A. Medication

- **Benzodiazepines** (e.g., Xanax, Klonopin) – short-term relief; risk of dependence.
- SSRIs (e.g., escitalopram, sertraline) – first-line long-term pharmacological treatment.
- Buspirone – non-addictive anxiolytic option.

B. Cognitive-Behavioral Therapy (CBT)

- Cognitive restructuring (challenging catastrophic thinking).
- Reducing information-processing biases.
- Applied relaxation training.
- Exposure to uncertainty.
- Problem-solving skills training.

CBT is considered **highly effective** and often preferred over long-term medication.

Case study: 189-190 & 194

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5. Panic Disorder

Key / Core Characteristics

- **Recurrent, unexpected panic attacks** ("out of the blue").

- Persistent concern about additional attacks or their consequences for **at least 1 month** (anticipatory anxiety).
- Significant maladaptive behavioral change related to attacks.

criteria for
Panic Disorder

DSM-5

A. Recurrent unexpected panic attacks. A panic attack is an abrupt surge of intense fear or intense discomfort that reaches a peak within minutes, and during which time four (or more) of the following symptoms occur:

Note: The abrupt surge can occur from a calm state or an anxious state.

1. Palpitations, pounding heart, or accelerated heart rate.
2. Sweating.
3. Trembling or shaking.
4. Sensations of shortness of breath or smothering.
5. Feelings of choking.
6. Chest pain or discomfort.
7. Nausea or abdominal distress.
8. Feeling dizzy, unsteady, light-headed, or faint.
9. Chills or heat sensations.
10. Paresthesias (numbness or tingling sensations).
11. Derealization (feelings of unreality) or depersonalization (being detached from oneself).
12. Fear of losing control or "going crazy."
13. Fear of dying.

Note: Culture-specific symptoms (e.g., tinnitus, neck soreness, headache, uncontrollable screaming or crying) may be seen. Such symptoms should not count as one of the four required symptoms.

B. At least one of the attacks has been followed by 1 month (or more) of one or both of the following:

1. Persistent concern or worry about additional panic attacks or their consequences (e.g., losing control, having a heart attack, "going crazy").
2. A significant maladaptive change in behavior related to the attacks (e.g., behaviors designed to avoid having panic attacks, such as avoidance of exercise or unfamiliar situations).

C. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism, cardiopulmonary disorders).

D. The disturbance is not better explained by another mental disorder (e.g., the panic attacks do not occur only in response to feared social situations, as in social anxiety disorder; in response to circumscribed phobic objects or situations, as in specific phobia; in response to obsessions, as in obsessive-compulsive disorder; in response to reminders of traumatic events, as in posttraumatic stress disorder; or in response to separation from attachment figures, as in separation anxiety disorder).

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Panic Attack Features

- Abrupt onset.
- Peak within **10 minutes**.
- Usually last **20–30 minutes**.
- At least 4 of 13 symptoms required.

Common Physical Symptoms:

- Palpitations
- Sweating
- Trembling
- Shortness of breath
- Chest pain
- Dizziness
- Nausea

Cognitive Symptoms:

- Fear of dying
- Fear of losing control/"going crazy"
- Depersonalization or derealization

2. Behavioural Elements

- Frequent emergency room visits (misinterpreting symptoms as heart attack).
- Avoidance of places associated with previous attacks.
- Development of **anticipatory anxiety**.
- Safety behaviors (carrying medication, sitting near exits).
- Possible development of agoraphobia.

3. Causes

A. Biological Factors

- Genetic vulnerability (runs in families).
- Hypersensitivity of the "fear network" in the brain (amygdala involvement).
- Dysregulation in norepinephrine and serotonin systems.
- Increased sensitivity to bodily sensations (anxiety sensitivity).

B. Psychological Factors

- Catastrophic misinterpretation of bodily sensations
  (e.g., "My heart is racing—I'm having a heart attack").
- Conditioning: panic attacks become associated with certain environments.
- Learned fear of internal sensations (interoceptive conditioning).

C. Social / Environmental Factors

- Major life stress.
- Childhood adversity.
- Health-related stress or serious illness.

4. Treatment

A. Medication

- SSRIs (first-line treatment).
- Benzodiazepines (short-term use).
- SNRIs as alternative options.

B. Cognitive-Behavioral Therapy (CBT)

- Psychoeducation about panic.
- Cognitive restructuring (challenging catastrophic thoughts).
- Interoceptive exposure (inducing bodily sensations safely).
- In vivo exposure (confronting avoided situations).
- Breathing retraining.

CBT is highly effective and reduces relapse risk.

Case study: Pg 179